FAX

To: **5122957070**
Company:
Fax: 5122957070
Phone:

From: **pinedam1**
Fax:
Phone:
E-mail:

NOTES:

Confidentiality Notice: Confidential Health Information Enclosed

The information in this fax is confidential and may contain information protected by law. This fax is intended to be reviewed only by the individual or organization named above. If you are not the intended recipient, you are hereby notified that any review, dissemination, or copying of this fax and its attachments is prohibited. If you have received this fax in error, please immediately notify the sender at the number listed above and arrange for the return or destruction of these documents.
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Date and time of transmission:
Number of pages including this cover sheet: 11

PAM HEALTH AT HOME

Fax

To: GULBIS, MARISSA	**From:** Pam Health at Home - Texas
Fax: 12104504995	**Pages:** 10
Phone:	**Date:** 02-25-2026 6:11 PM
Re: Order(s) for Signature	

Comments: URGENT: ATTENTION MD MCGINITY

Please review, sign, date and return asap, today if possible.

Claudia M. Orders Team
ph 210-728-9736
fax 512-295-7070

This fax contains orders for signature.
Please have the physician/clinician noted on this order.
Please sign, date, and return all pages to 1-512-295-7070.

PAM Health at Home – Texas
825 Main Street, Suite 110, Buda, TX 78610
Ph: 512-295-7000; Fax: 512-295-7070

IMPORTANT - PLEASE NOTE: The information transmitted herein is intended only for the person or entity to which it is addressed and may contain confidential, proprietary, protected, and/or privileged material. Any review, retransmission, disclosure, dissemination or other use of this information by persons or entities other than the intended recipient is prohibited. If you received this in error please contact the sender and destroy this facsimile.

Thank you!

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Order No:485-24329

HOME HEALTH CERTIFICATION AND PLAN OF CARE

1. Patient's HI Claim No.	2. Start Of Care Date	3. Certification Period	4. Medical Record No.	5. Provider No.
144705849	01/15/2026	01/15/2026 - 03/15/2026	8869	262009456

6. Patient's Name and Address
Martin, Melonese (8869)
13813 PURSELY RIDGE
San Antonio, TX 78245
Phone: (808) 630-3862

7. Provider's Name, Address and Telephone Number
PAM Health at Home
825 Main St Ste 110
Buda, TX 78610-3273
Phone: (512) 295-7000, Fax: (512) 295-7070

8. Date of Birth: 03/20/1973 **9. Sex:** ☐ M ☒ F

10. Medications: Dose/Frequency/Route (N)ew (C)hanged

Medication	Dose	Frequency	Route	Status
Cholecalciferol (vitamin D3) 50 mcg (2,000 unit) capsule [Amount Of/ Dosage Form: 1 Capsule]	2000units	daily	oral	LS
Colace 100 mg capsule [Amount Of/ Dosage Form: 2 Capsule]	100mg	daily	oral	LS
Duloxetine 20 mg capsule,delayed release [Amount Of/ Dosage Form: 1 Capsule]	20mg	daily	oral	LS
Folic acid 1 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	1mg	daily	oral	LS
Gabapentin 600 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	600mg	2 times a day	oral	LS
Lidocaine 5 % topical patch [Amount Of/ Dosage Form: 1 Patch]	5%	daily	topical	LS
Meloxicam 15 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	15mg	daily	oral	LS
Methocarbamol 500 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	500mg	4 times a day	oral	LS
Modafinil 200 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	200mg	daily	oral	LS
Sennosides 17.2 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	17.2mg	at bedtime	oral	LS
Tramadol 50 mg tablet [Amount Of/ Dosage Form: 1 Tablet]	50mg	every 4 hours as needed	oral	LS - PRN/As Needed

11. ICD-9 / 10 CM **Principal Diagnosis**

Code	Code Description	Date	O/E
Z47.89	Encounter for other orthopedic aftercare		

12. ICD-9 / 10 CM **Surgical Procedure**

Code	Code Description	Date	O/E

13. ICD-9 / 10 CM **Other Pertinent Diagnosis**

Code	Code Description	Date	O/E

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M48.03	Spinal stenosis, cervicothoracic region
G89.18	Other acute postprocedural pain
M43.24	Fusion of spine, thoracic region
D75.A	Glucose-6-phosphate dehydrogenase (G6PD) deficiency without anemia
E16.2	Hypoglycemia, unspecified
K21.9	Gastro-esophageal reflux disease without esophagitis
Z79.891	Long term (current) use of opiate analgesic

Melonese Martin MRN: 61944354 Enc#
DOB: 3/20/1973 HPh#808-630-3862
TRICARE HEALTHCARE ALLIANCE
DOS: 6/24/26
Michael J McGinity, MD
UTH MARC, NEUROSURGERY-UT Med
Ref: McGinity, Michael J MD 210-450-9060

14. DME and Supplies:
DME: Elevated Toilet Seat, Tub/Shower Bench, Walker
PPE MASK, PROBE COVERS, GLOVES

15. Safety Measures:
911 Emergency / Protocol, C- Spine Precaution, Emergency Plan Developed, Equipment Safety, Fall / Safety Precautions, Fall Precautions, Infection Control Standard Measures and Precautions, Keep Pathways Clear, Keep Pathways Clear and Safe, OPIOID PRECAUTIONS, Safe in Movements, Safe Transfers, Safety in ADLs, Slow Position Change, Standard Precautions/Infection Control, Support During Transfer and Ambulation, Transfer Precautions, Universal Precautions, Use of Assistive Devices, Walker / Cane

16. Nutritional Req.
No Concentrated Sweets,
Heart healthy

17. Allergies:
Medication Allergies: aspirin, codeine, Sulfa (Sulfonamide Antibiotics), Tylenol-Codeine

18.A. Functional Limitations
☐ 1- Amputation ☐ 4- Hearing ☑ 7- Ambulation
☑ 2- Bowel/Bladder (Incontinence) ☐ 5- Paralysis ☐ 8- Speech
☐ 3- Contracture ☑ 6- Endurance ☐ 9 - Legally Blind
☐ A- Dyspnea with minimal exertion
☐ B- Other (Specify):

18.B. Activities Permitted
☐ 1- Complete Bedrest ☑ 5- Exercises Prescribed ☐ 9- Cane
☐ 2- Bedrest BRP ☐ 6- Partial Weight Bearing ☐ A- Wheelchair
☑ 3- Up as Tolerated ☐ 7- Independent at Home ☑ B- Walker
☐ 4- Transfer Bed/Chair ☐ 8- Crutches ☐ C- No Restrictions
☐ D- Other (Specify):

19. Mental Status
☑ 1 - Oriented ☐ 2 - Comatose ☐ 3 - Forgetful ☐ 4 - Depressed ☐ 5 - Disoriented ☐ 6 - Lethargic ☐ 7 - Agitated
☐ 8 - Other(Specify):

20. Prognosis: ☐ 1- Poor ☐ 2- Guarded ☑ 3- Fair ☐ 4- Good ☐ 5- Excellent ☐ 6- Other (Specify):

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21. Treatments and Orders for Disciplines:

Frequency
SN Frequency: 1W1, 1EOW8,DC OT ORDER,PT Frequency: 1W4 Starting Week of 01/15/2026

Initial Order received:
Yes

Vitals Normal Range
BP Systolic: 90 - 160
BP Diastolic: 56 - 100
Pulse: 56 - 100
Respiratory: 12 - 28
Temperature: 96.5 - 100.5
O2 Saturation: 90 - 100

Vitals Physician Alert Range
Temperature: < 96.5 or > 100.5
Pulse: < 56 or > 100
Respiratory: < 12 or > 28
BP Systolic: < 90 or > 160
BP Diastolic: < 56 or > 100
O2 Saturation: < 90
Fasting Blood Sugar: < 70 or > 300
Random Blood Sugar: < 70 or > 350
Pain: > 7

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Ref: McGinity, Michael J, MD 210-450-9060

SN

BEST PRACTICE FALL PREVENTION:
- Clinician will perform fall assessment and instruct patient /caregiver on fall prevention.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION:
- Clinician to perform drug regimen review/assessment to assess for potential clinically significant medications issues.
- Clinician to report potentially clinically significant medications issues to physician within 1 calendar day.
- Clinician to assess/instruct on high risk and other medications to include effects, potential adverse effects, interactions and duplicate therapy, dose route and frequency.

BEST PRACTICE MONITOR AND MITIGATE PAIN:
- Clinician will instruct patient /caregiver on methods to mitigate pain to include medications and non-pharmacological interventions to decrease pain.

BEST PRACTICE PRESSURE ULCER PREVENTION:
- Clinician will perform validated test (BRADEN)to assess patient for risk of development of Pressure ulcers.
- Clinician will perform teaching and training to patient/caregiver for pressure ulcer prevention.

GENITOURINARY:
- Clinician to obtain Urine for U/A and C& S as needed for s/s of UTI via clean catch and notify physician of results.

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HOSPITALIZATION RISK FACTORS:

- Clinician will instruct the patient and/or caregivers on the following unstable disease processes that may lead to unplanned hospitalizations: C3-6, c2-T1 FUSION, C7-T1 DECOMPRESSION, LUMBAR DRAIN R/T CSF LEAK, C7-T1 ACDF AND C7-T6 EXTENSION, SPINAL STENOSIS, GERD
- Clinician will instruct patient/caregiver on ways to prevent unintentional weight loss and poor nutrition (high calorie foods, recording (wt/diet), etc
- Clinician will instruct patient and/or caregivers on polypharmacy, duplicate medications, adverse effects, et and compliance/understanding of medication regimen.
- Clinician will assess and teach patient and/or caregivers regarding declines in mental, emotional, or behavior status and when to report to the MD.
- Clinician to instruct patient and/or caregivers on ways to avoid exhaustion and fatigue to include adequate sleep, nutrition, exercise, etc.
- Clinician will instruct patient and/or caregivers on fall prevention to include clear pathways, adequate lighting, assistive devices, slow change in position, etc.

OTHER

- Monitor surgical sites anterior cervical region and posterior thoracic region for s/s of infection or complications. Report any abnormalities to MD.

PT

PHYSICAL THERAPY:

- Evaluation and Treatment
- Establish Home Exercise Program and Activity Program
- Home safety evaluation and fall prevention/teach fall risk reduction strategies
- Therapeutic Exercises: Neuromotor training/neuromuscular re-education
- Therapeutic Exercises: Body Mechanics, postural training and positioning
- Therapeutic Exercises: Strength training
- Gait Training: Asst device (specify) ___LRD_____
- Gait Training: Even and uneven surfaces and/or natural terrain
- Transfer Training: Sit <-> Stand

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Assessment Summary

FOCUS OF CARE: C3-6, c2-T1 FUSION, C7-T1 DECOMPRESSION, LUMBAR DRAIN R/T CSF LEAK, C7-T1 ACDF AND C7-T6 EXTENSION

CLINICAL SUMMARY: PATIENT IS A 52 Y/O FEMALE WITH DX OF SPINAL STENOSIS, G6PD DEFICIENCY, BLE WEAKNESS, GERD, VIT D DEFICIT, MUSCLE SPASMS, HYPOGLYCEMIC DISORDER, NEUROPATHIC PAIN, DMII (denies), HTN, AND NARCOLEPSY. Her surgical history as follows: Laminectomy C3-6, c2-T1 FUSION, C7-T1 DECOMPRESSION, LUMBAR DRAIN R/T CSF LEAK, C7-T1 ACDF AND C7-T6 EXTENSION.PATIENT LIVES ALINE IN A 2 STORY HOME WITH HER DOG. IT IS UNCLEAR AT THIS TIME IF HER FRIEND IS WILLING AND OR ABLE TO ASSIST WITH ADL/IADL'S. PATIENT IS IN AGREEMENT TO HH SERVICES AND POC. CONSENTS WERE OBTAINED AND HEAD TO TOE ASSESSMENT PERFORMED. HER SURGICAL SITES ARE OPEN TO AIR AND CLOSED. WILL CONTINUE TO MONITOR RECENTLY CLOSED SITES FOR COMPLICATIONS. PATIENT HAS F/U APPT ON 2/18/26. PT LEFT IN STABLE

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CONDITION AND IN GOOD SPIRITS FOLLOWING SNV. SOC REPORTED TO SHARON FOR DR MICHEAL MCGINITY AT 11:06am.

Frequency may be completed with both face to face visits and/or telehealth (if appropriate to meet patient treatment needs).
All qualified disciplines may do non-billable visits to supervise staff and collect oasis data as needed.
Within 5 days of the end of episode, a qualified professional may have a PRN visit to complete a comprehensive/OASIS assessment to establish the need for continuing home health services or discharge.
May accept orders from all physicians caring for the patient.
Any qualified disciplines may perform visit for resumption of care when patient discharged home from inpatient facility.
May resume all care prior to the hospitalization.
PAM Health at Home may discharge patient if remains in facility when certification period expires.
All skilled disciplines may check oxygen saturation prn sob.

I certify that the patient is confined to his/her home and needs intermittent skilled nursing, physical therapy, occupational therapy and/or speech therapy for the next 60 days.
I attest to the following:
1. MELONESE MARTIN is under my care.
2. I have received the following patient records from PAM Health at Home:
a. Patient Summary Form
b. Hospital/LTC stay information (if applicable)
c. Home Health Plan of Care
d. And the signature below is my signature
e. These patient records were considered in advance of my certification of the patient's eligibility for home health services under Medicare.
I certify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy for the next 60 days. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter has occurred on 12/23/2025 by SHANTE SPAULDING, NP and it is related to the primary reason the patient requires home health services. PAM HEALTH AT HOME, HOME HEALTH AGENCY, MAY PERFORM SOC ON 1/15/26.

Risk of Hospitalization

Risks: Multiple hospitalizations (2 or more) in the past 6 months; Multiple emergency department visits (2 or more) in the past 6 months; Reported or observed history of difficulty complying with any medical instructions (for example, medications, diet, exercise) in the past 3 months; Currently taking 5 or more medications;
Interventions: Referrals - SN,PT,OT; Medication Management - Reconciliation, Assess patients: Knowledge,Ability,Resources & Adherence, Education; Patient/Family Education; Phone Monitoring; Fall Prevention Program; Individualized Patient Emergency Care Plan; Care Coordination (Physicians, Hospitals, Nursing homes...)
Risk Factors: Discharged from Hospital or Skilled Nursing Facility (M1000); More than 2 Secondary Diagnoses (M1022 & M1024); Lives Alone (M1100) ; Inadequate Support Network (M1100); Dyspnea (M1400) ; ADL Assistance Needed (M2102 & M2110)

Psychosocial Assessment

N/A

Cognitive Status

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Cognitive status: No behaviors demonstrated

Advance Directive

Patient/Caregiver reports no Advanced Directives in place
Patient does not have an advance directive;
Have an Advanced Care Plan and it is documented in the medical record: No - Did not wish to provide an ADC or name a SDM;
Have a Surrogate Decision Maker and information documented in the medical record: No - Did not wish to provide an ADC or name a SDM;

Homebound Status

Patient Homebound Reasons
- Is the Patient Homebound: Yes
- Homebound Reasons: Need assistance for all activities; Residual weakness; Require assistance to ambulate; Unable to safely leave home unassisted; Severe SOB, SOB upon exertion; Dependent upon adaptive device(s);
- Patient Chronic and Stable: Yes
- Patient leaving the home require Considerable and Taxing effort : Yes
- Comments: Pt is homebound due to having Multiple medical problems and functional impairment, the patient requires use of assistive
devices and the assistance of another person in order to leave home. It requires considerable effort that is very taxing on the patient to leave home.

22. Rehabilitation Potential/Discharge Plans/Goals :

Rehab Potential

Fair to achieve stated goals with skilled interventions and patient's compliance with plan of care.

Discharge Plan

Discharge when medical condition is stable and patient is no longer in need of skilled services

Goals

SN

BEST PRACTICE FALL PREVENTION: - LTG

- LTG-Patient/caregiver will verbalize understanding of interventions to prevent falls by end of episode.

BEST PRACTICE FALL PREVENTION: - STG

- STG-Patient/caregiver will verbalize 2 interventions to prevent falls by week 4. Patient will demonstrate decrease fall risk as evidenced by the tug test or MACH 10 by week 4.

BEST PRACTICE MEDICATIONS/DRUG EDUCATION: - LTG

- LTG-Patient /caregiver will be able to name s/s of adverse side effects to report to MD by end of episode.
- LTG-Patient /caregiver will be able to verbalize/demonstrate correct dose, route, and frequency of all medications and will be able to recall adverse effects to report to MD by end of episode.

BEST PRACTICE MONITOR AND MITIGATE PAIN: - LTG

- LTG-Patient/caregiver will demonstrate decreased pain with ambulation as evidenced by increased activity tolerance by end

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of episode.

BEST PRACTICE PRESSURE ULCER PREVENTION: - LTG
- LTG-Patient/Caregiver will demonstrate/verbalize understanding of methods to prevent pressure ulcers by end of episode.

SURGICAL WOUND: - STG
- STG-Patient/Caregiver will be able to identify 2 s/s of infection of surgical wound by week 4

PATIENT STATED GOALS: - LTG
- LTG-***Patient stated LTG **** strengthen leg get back to driving and live independently

HOSPITALIZATION RISK FACTORS: - LTG
- Patient and/or caregivers will verbalize understanding of ways to mitigate risk of return to the hospital and unplanned ER use.

PT

HEP: - STG
- STG-Patient will demonstrate independent adherence to cervical precautions during all functional mobility tasks.

MOBILITY/GAIT: - LTG
- LTG-Patient will demonstrate independent adherence to cervical precautions during all functional mobility tasks.
- LTG-Patient will ambulate =200 feet in the home independently with safe gait mechanics while maintaining neutral cervical alignment.

THERAPEUTIC EXERCISES: - LTG
- LTG-Patient will perform prescribed B UE and B LE strengthening exercises independently without compromising cervical restrictions.

TRANSFER TRAINING: - LTG
- LTG-Patient will complete sit-to-stand transfers independently without cervical motion or violation of precautions.
- LTG-Patient will demonstrate functional independence and safety appropriate for transition to outpatient physical therapy per MD clearance.

23. Clinician's Signature and Date of Verbal SOC Where Applicable: (Electronically Signed)
Verbal SOC Date: 01/15/2026
Signed By: Percival, Synina - RN on 01/15/2026 04:37 PM, Cosigned by Strait, Donna - RN

25. Date HHA Received Signed POT:

24. Physician's Name and Address
MARISSA GULBIS PA-C
8300 Floyd Curl Dr, Suite 7A
San Antonio, TX 78229,NPI: 1881129658,Phone: (210) 450-9060, Fax: (210) 450-4995

26. I certify/ recertify that this patient is confined to his/her home and needs intermittent skilled nursing care, physical therapy and/or speech therapy or continues to need occupational therapy. This patient is under my care, and I have authorized the services on this plan of care and I or another physician will periodically review this plan. I attest that a valid face-to-face encounter occurred (or will occur) within timeframe requirements and it is related to the primary reason the patient requires home health service

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27. Attending Physician's Signature and Date Signed:

3/3/26

28. Anyone who misrepresents,falsifies, or conceals essential information required for payment of Federal funds may be subject to fine, imprisonment, or civil penality under applicable Federal laws.

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